PROVIDER REFERENCE

SETD5 Syndrome: *For Clinical Visit Use*

setd5syndrome.live

WHAT IS SETD5 SYNDROME?

SETD5 Syndrome is a rare genetic condition caused by a variant (mutation or deletion) in the SETD5 gene on chromosome 3 (3p25.3). The SETD5 gene plays a role in regulating how other genes are switched on and off during development. When one copy is missing or not working, gene regulation is disrupted, resulting in haploinsufficiency. Most cases arise de novo (not inherited), but it can be inherited. The condition affects each person differently; no two individuals share the exact same presentation.

FEATURES REPORTED IN SETD5 SYNDROME

Not every individual will have all features listed. Severity varies widely, even among people with the same variant type.

NEURODEVELOPMENTAL & BEHAVIORAL

- Intellectual disability (mild to moderate in most)
- Global developmental delay
- Speech and language delay (nearly universal); expressive more affected than receptive
- ADHD features: inattention, hyperactivity, impulsivity
- Autism spectrum features in some
- Anxiety; behavioral rigidity; sensory sensitivities
- Repetitive movements or behaviors
- Working memory and short-term memory difficulties
- Affectionate, sociable disposition common

NEUROLOGICAL, PHYSICAL & GROWTH

- Hypotonia (low muscle tone); gait abnormalities including toe-walking
- Epilepsy or seizures in some; EEG abnormalities
- Short stature; feeding difficulties in infancy
- Triangular face; hypertelorism; synophrys; depressed nasal bridge
- High-arched or cleft palate; micrognathia
- Congenital heart defects (various types) in some
- Eye anomalies: strabismus, nystagmus, ptosis
- Hearing impairment in some
- Scoliosis, kyphosis, joint laxity; reduced bone mineral density

FOR THIS APPOINTMENT

Questions drawn from published research, clinician observations, and experiences reported by families across the broader SETD5 Syndrome community.

FOR MEDICAL PROVIDERS

- Should we pursue a genetics consultation if we haven't already?
- Given the risk of seizures, should we see a neurologist? Would an EEG or MRI be appropriate?
- Should we have cardiology, ophthalmology, and audiology screenings?
- Should we see an orthopedic specialist to monitor spine and bone mineral density?
- How should we monitor growth? Would a developmental pediatrician help coordinate care?
- How does SETD5 Syndrome relate to what we're currently seeing behaviorally?
- Are there genetic counseling resources for our extended family?
- What should we watch for as our child gets older?
- Can you help us understand how to access early intervention or therapy services?

FOR EDUCATORS & SUPPORT STAFF

- Is our child eligible for an IEP or 504 Plan?
- Should speech-language therapy be part of their support plan?
- Would occupational and/or physical therapy be appropriate?
- How can we support a 'spiky' cognitive profile — building on strengths while addressing challenges?
- Can we build in predictable routines and minimize transitions to support regulation?
- Would AAC (augmentative and alternative communication) be appropriate for our child?
- What sensory accommodations might help?

GENE	INHERITANCE	PREVALENCE	OMIM	REGISTRY
SETD5 (chromosome 3p25.3)	Typically de novo; can be inherited (AD)	Fewer than 100 in published literature; ~1,100 in patient community	#615761	Simons Searchlight

More information, plain-language research summaries, and a medical terms guide are available at setd5syndrome.live. This document is for informational purposes only and does not constitute medical advice. Always consult your child's medical team for guidance specific to your situation. Prepared by setd5syndrome.live — a family-created resource.

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